



Care Indeed Home Health

Mock Patient Evaluation Document · Version 1.0 MOCK · Training Date
July 1, 2026

MOCK TRAINING RECORD
SYNTHETIC UAT DATA
NOT FOR CLINICAL USE
DO NOT UPLOAD TO LIVE PATIENT
CHART

EVALUATION 1 — HIGH ACUITY

Primary Diagnosis: Acute Exacerbation of Congestive Heart Failure (NYHA Class III)

PATIENT DEMOGRAPHICS

Field	Detail
Patient Name	Margaret Louise Finnegan
Date of Birth	03/14/1941
Age	84
Sex	Female
Gender Identity	Female (she/her)
Race/Ethnicity	White / Non-Hispanic
Primary Language	English
Interpreter Needed	No
Marital Status	Widowed
Social Security # (last 4)	XXX-XX-7823
Medical Record Number	CI-2025-00417
Address of Service	1847 Oleander Court, Unit B, San Mateo, CA 94401
Home Phone	(650) 555-0193
Cell Phone	(650) 555-0247
Email	None
County	San Mateo

EMERGENCY CONTACTS

Priority	Name	Relationship	Phone (Primary)	Phone (Alt)	Lives with Patient?
1	Robert Finnegan	Son	(650) 555-0381	(415) 555-0294	No — lives 12 miles away
2	Diana Finnegan-Marsh	Daughter	(510) 555-0762	(510) 555-0148	No — lives 38 miles away
3	Patricia Yuen	Neighbor / Friend	(650) 555-0455	N/A	No — lives next door, Unit A

LIVING SITUATION

Field	Detail
Lives With	Alone
Residence Type	Single-story duplex, rented
Stairs to Enter	2 steps to front door (no handrail)
Interior Stairs	None
Elevator	N/A
Pets	1 indoor cat ("Biscuit") — elderly, not aggressive
Home Access Instructions	Lockbox on front door handle; code to be provided to care team by son Robert
DME in Home	Hospital bed (rented), bedside commode, rolling walker, portable oxygen concentrator (5 LPM)

PAYER INFORMATION

Priority	Payer	ID / Policy Number	Group Number	Effective Date
Primary	Medicare Part A (Traditional)	1EG4-TE5-MK72	N/A	07/01/2014
Secondary	Medi-Cal (Fee-for-Service)	94038712A	N/A	01/01/2019
Tertiary	N/A			

Private Pay: No Workers' Comp: No VA/TRICARE: No

PHYSICIAN INFORMATION

Role	Name	Specialty	Phone	Fax	NPI	Address
Attending / Certifying Physician	Dr. Alan K. Chowdhury	Internal Medicine	(650) 555-0612	(650) 555- 0613	1234567890	2100 Bay Medical Plaza, Ste 310, San Mateo, CA 94401
Cardiologist	Dr. Renata Silva-Gutierrez	Cardiology	(650) 555-0820	(650) 555- 0821	2345678901	2100 Bay Medical Plaza, Ste 445, San Mateo, CA 94401
PCP (if different)	Same as attending					

Face-to-Face Encounter Date: 06/18/2025 F2F Performed By: Dr. Alan K. Chowdhury F2F Encounter Findings: Patient seen in office; significant dyspnea on minimal exertion (walking >15 ft), bilateral lower extremity edema (+2 pitting), weight gain 8 lbs in 10 days, oxygen saturation 89% on room air at rest. Requires skilled nursing for CHF management, medication teaching, daily weight monitoring, and oxygen management. Homebound due to severe dyspnea and functional limitation.

REFERRAL / HOSPITALIZATION HISTORY

Event	Facility	Dates	Reason
Hospital Discharge	San Mateo Medical Center	06/12/2025 – 06/17/2025	Acute CHF exacerbation, fluid overload, IV diuresis
ED Visit	San Mateo Medical Center	05/01/2025	Syncope — evaluated and released
Hospital Admission	San Mateo Medical Center	02/08/2025 – 02/12/2025	CHF exacerbation, pneumonia
Hospital Admission	Sequoia Hospital	11/15/2024 – 11/19/2024	CHF exacerbation with atrial fibrillation RVR

Start of Care Date: 06/19/2025 Referral Source: San Mateo Medical Center Discharge Planning — Case Manager: Linda Torres, RN, (650) 555-0399

DIAGNOSES

Priority	ICD-10 Code	Diagnosis	Onset
Primary	I50.33	Acute on chronic diastolic (congestive) heart failure, NYHA Class III	06/12/2025
Secondary	I48.91	Unspecified atrial fibrillation (chronic, rate-controlled)	2018
Secondary	I10	Essential (primary) hypertension	2005
Secondary	E11.65	Type 2 diabetes mellitus with hyperglycemia	2010
Secondary	N18.3	Chronic kidney disease, stage 3 (GFR 38 mL/min, 06/2025)	2020
Secondary	E78.5	Dyslipidemia, unspecified	2008
Secondary	J44.1	Chronic obstructive pulmonary disease with acute exacerbation (resolved, history of)	2022
Secondary	M81.0	Age-related osteoporosis without current pathological fracture	2019
Secondary	G47.33	Obstructive sleep apnea	2017
Secondary	F32.1	Major depressive disorder, single episode, moderate	2023
Secondary	H25.13	Age-related nuclear cataract, bilateral	2021

Surgical History:

Right total knee arthroplasty (2016)

Cholecystectomy (2003)

Hysterectomy, total abdominal (1998)

Cataract extraction, left eye (2022)

ALLERGIES

Allergen	Type	Reaction	Severity
Penicillin	Drug	Anaphylaxis (throat swelling, hypotension) — documented 1987	SEVERE
Sulfa drugs	Drug	Diffuse maculopapular rash, pruritus	Moderate
Iodine contrast dye	Contrast	Urticaria, facial swelling	Moderate
Latex	Environmental	Contact dermatitis	Mild
Shellfish	Food	Nausea, abdominal cramping	Mild

NKDA: ☐ No known drug allergies — NOT applicable; see above

CURRENT MEDICATIONS (as of discharge 06/17/2025)

#	Medication	Dose	Route	Frequency	Prescriber	Purpose	High-Risk?
1	Furosemide (Lasix)	80 mg	PO	BID (8 AM, 2 PM)	Chowdhury	Diuresis / CHF	Yes
2	Potassium Chloride (K-Dur)	20 mEq	PO	BID	Chowdhury	K+ replacement (with diuretic)	Yes
3	Carvedilol (Coreg)	12.5 mg	PO	BID	Silva-Gutierrez	Heart failure / rate control	Yes
4	Lisinopril	10 mg	PO	Daily (AM)	Chowdhury	HTN / CHF / renal protection	Yes
5	Warfarin (Coumadin)	4 mg	PO	Daily (6 PM)	Silva-Gutierrez	Anticoagulation (A-fib)	YES — HIGH ALERT
6	Digoxin (Lanoxin)	0.125 mg	PO	Daily (AM)	Silva-Gutierrez	A-fib rate control / CHF	YES — HIGH ALERT
7	Metformin	500 mg	PO	BID (with meals)	Chowdhury	Type 2 DM	Yes
8	Atorvastatin (Lipitor)	40 mg	PO	Daily (HS)	Chowdhury	Dyslipidemia	No
9	Sertraline (Zoloft)	100 mg	PO	Daily (AM)	Chowdhury	Depression	No
10	Albuterol (ProAir HFA)	2 puffs	INH	Q4-6H PRN	Chowdhury	Bronchospasm / COPD	No
11	Alendronate (Fosamax)	70 mg	PO	Weekly (Sunday AM, 30 min before food/meds)	Chowdhury	Osteoporosis	No
12	Calcium + Vitamin D	600 mg / 400 IU	PO	BID	Chowdhury	Bone health	No
13	Docusate Sodium (Colace)	100 mg	PO	BID	Chowdhury	Constipation prophylaxis	No
14	Acetaminophen (Tylenol)	650 mg	PO	Q6H PRN	Chowdhury	Pain (knee, general)	No
15	Oxygen	2-3 LPM	Nasal cannula	Continuous at rest; 3 LPM with activity	Chowdhury	Hypoxemia / CHF / COPD	Yes

Medication Reconciliation Completed: ☐ Yes — by admitting RN on _____ Discrepancies Identified: ☐ Yes ☐ No (if yes, document and notify physician) Controlled Substances in Home: None Insulin in Home: No Sharps in Home: No Medication Organizer Used: Yes — 7-day AM/PM pill organizer (filled by son Robert weekly)

CLINICAL ASSESSMENT — VITAL SIGNS AT ADMISSION

Parameter	Value	Date/Time	Normal Range	Abnormal?
Blood Pressure	158/92 mmHg	06/19/2025 10:30 AM	<130/80	Yes — elevated
Heart Rate	88 bpm, irregularly irregular	06/19/2025 10:30 AM	60-100	WNL (irregular rhythm — known A-fib)
Respiratory Rate	22 breaths/min	06/19/2025 10:30 AM	12-20	Yes — mildly elevated
Temperature	98.2°F (oral)	06/19/2025 10:30 AM	97.0-99.0	WNL
Oxygen Saturation (SpO2)	91% on 2 LPM O2	06/19/2025 10:30 AM	≥95%	Yes — low
SpO2 on Room Air	87% (desaturated within 45 seconds of O2 removal)	06/19/2025 10:35 AM	≥95%	Yes — significantly low
Weight	178 lbs (80.7 kg)	06/19/2025 10:25 AM	Baseline ~165 lbs pre-exacerbation	Yes — 13 lbs above baseline
Height	5'4" (162.5 cm)	Per history		
BMI	30.5	Calculated	18.5-24.9	Yes — obese class I
Pain	4/10 (bilateral knee pain, worse with standing)	06/19/2025	0 = no pain	Yes
Blood Glucose	188 mg/dL (fasting)	06/19/2025 08:15 AM (patient report)	70-130 fasting	Yes — elevated

CLINICAL ASSESSMENT — SYSTEMS REVIEW

Cardiovascular:

History of CHF (diastolic, NYHA Class III at present), chronic atrial fibrillation (rate-controlled), hypertension. Currently on anticoagulation with warfarin; INR due for check (last INR 2.3 on 06/16/2025 — inpatient). Bilateral lower extremity edema: +2 pitting, bilateral, to mid-shin. Skin intact, no weeping. JVD noted at 45 degrees. S3 gallop appreciated on auscultation. Peripheral pulses: dorsalis pedis 1+ bilaterally, posterior tibial 1+ bilaterally. Capillary refill: 4 seconds bilaterally (delayed). No calf tenderness, no Homans' sign. Daily weight monitoring ordered — patient has digital scale in bathroom.

Respiratory:

History of COPD (mild-moderate, FEV1 62% predicted on last PFT 2023). Oxygen-dependent at rest (2 LPM) and with activity (3 LPM).

Bilateral basilar crackles (fine), more prominent on right than left — consistent with residual fluid.
Dyspnea on exertion: onset with walking >10-15 feet; relieved with rest and supplemental O2 increase.
Dyspnea at rest: intermittent, particularly when supine (orthopnea — sleeps with 3 pillows elevated).
Productive cough: small amounts of white/frothy sputum, non-purulent.
No hemoptysis.
CPAP for OSA — patient reports non-compliance x 6 months ("the mask bothers me").

Neurological / Cognitive:

Alert and oriented x4 (person, place, time, situation).
MMSE-equivalent screening: 26/30 (missed serial 7s, missed date by 1 day, difficulty with 3-item recall at 5 minutes — 1/3 recalled).
Speech clear, appropriate, coherent. No aphasia or dysarthria.
Cranial nerves grossly intact.
Bilateral upper extremity strength 4/5 (mild generalized weakness post-hospitalization).
Bilateral lower extremity strength 3+/5 (weakness, deconditioning, knee pain).
Sensation intact to light touch bilaterally — diminished at toes bilaterally (diabetic neuropathy suspected).
No tremor, no focal deficits.
Visual acuity: diminished (cataracts bilateral, L > R; uses reading glasses). Difficulty reading medication bottles.
Hearing: mild bilateral sensorineural hearing loss — no hearing aids, denies need but frequently asks for repetition.

Gastrointestinal:

Abdomen soft, non-tender, non-distended. Bowel sounds active x4 quadrants.
Last BM: 06/18/2025 — soft, brown, no blood.
Dentures: upper plate (fits well); lower natural teeth (several missing).
Appetite: poor since hospitalization; estimates eating 40-50% of meals.
Diet order: low-sodium (2g Na+/day), diabetic-consistent, fluid restriction 1.5L/day.
Reports difficulty preparing meals; son Robert brings meals 3x/week, otherwise relies on frozen meals (high sodium concern).

Genitourinary:

Voiding independently; nocturia x3-4 (likely related to diuretic timing and CHF).
No incontinence. No dysuria, no hematuria.
No catheter. No history of UTI in past 12 months.
CKD Stage 3 — last creatinine 1.8, BUN 34, GFR 38 (06/15/2025 inpatient labs).
Nephrology referral pending.

Integumentary / Skin:

Skin generally intact.
Bilateral lower extremities: +2 pitting edema, skin taut and shiny, no breakdown, no cellulitis. Hemosiderin staining bilateral medial ankles (chronic venous changes).
Sacrum/coccyx: intact, no redness, no pressure injury. Braden Scale: 16 (mild risk).
Bilateral knees: well-healed surgical scar right knee (TKA 2016); no erythema, warmth, or drainage.
No wounds, no ulcers, no skin tears.
Skin turgor: mildly decreased (upper extremities) — may indicate suboptimal hydration despite fluid overload.

Musculoskeletal / Functional:

Transfers: bed to chair with moderate assist of 1 and use of bedrail. Stands with rolling walker and moderate assist of 1.
Ambulation: rolling walker, 10-15 feet maximum before significant dyspnea and fatigue. Requires contact guard to standby assist.
Stairs: unable at this time (2 steps to front door — uses single handrail + assist of 1).
ADLs:
Bathing: requires moderate assist (seated shower, uses shower chair)

Dressing upper body: minimal assist (difficulty with overhead motion due to fatigue)
Dressing lower body: moderate assist (cannot bend to put on shoes/socks due to edema, dyspnea)
Toileting: uses bedside commode at night; ambulates to bathroom with walker during day (standby assist)
Eating: independent (when food is prepared and placed within reach)
Grooming: independent (seated)
IADLs:
Meal preparation: unable (fatigue, dyspnea, standing tolerance <3 minutes)
Medication management: requires supervision/assist (poor understanding of regimen changes post-discharge; visual impairment limits label reading)
Telephone use: independent
Laundry: unable
Light housekeeping: unable
Shopping: unable
Transportation: unable to drive; dependent on son or paratransit

Psychosocial / Emotional:

PHQ-9 Score: 14 (moderate depression)
Reports feeling "useless" and "like a burden to my kids."
Widowed x 7 years; reports ongoing grief, worsened with repeated hospitalizations.
Social isolation: lives alone, limited visitors (son 2-3x/week; daughter monthly). Neighbor Patricia checks in daily by phone.
Previous psychotherapy: none. Open to MSW support "if it would help."
Denies suicidal ideation, homicidal ideation, or self-harm.
Denies alcohol, tobacco (quit 1995), or illicit drug use.
Spiritual: Catholic; parish priest visits monthly. Faith is important to her.
Reports anxiety about falling when alone, especially at night.

HOMEBOUND STATUS ASSESSMENT

Criterion	Finding
Criterion 1 — Considerable and taxing effort to leave home	MET. Patient develops significant dyspnea (SpO2 drops to 87% on RA) with ambulation >10-15 feet. Requires rolling walker + assist of 1 to ambulate. Unable to navigate 2 steps to front door without assist. Severe fatigue with minimal activity. Bilateral LE edema limits mobility.
Criterion 2 — Absences from home are infrequent/short/for medical care	MET. Patient has not left home since hospital discharge 06/17/2025. Son drives her to medical appointments only. No other outings planned or feasible.
Homebound Status	CONFIRMED — homebound

ADVANCE DIRECTIVES

Document	Status	On File?	Date Executed
Advance Health Care Directive (AHCD)	Yes — executed	☐ Copy provided at admission	03/2021
Power of Attorney for Health Care	Robert Finnegan (son) designated as agent	Included in AHCD	03/2021
POLST	Yes — completed	☐ Copy provided at admission	06/2025 (updated during hospitalization)
POLST Orders	Full treatment (Section A); selective treatment including hospitalization but no intubation (Section B); trial period of artificial nutrition (Section D)		
DNR	No — patient is FULL CODE per POLST Section A		
Mental Health Directive	None		

HOME SAFETY ASSESSMENT

Area	Finding	Recommendation	Patient Response
Entry	2 steps, no handrail on left side	Install grab rail left side of steps	Son Robert will install this weekend
Lighting — Entry	Dim; single 40W bulb	Replace with higher-wattage LED bulb	Agreed
Lighting — Hallway to Bathroom	Adequate	None	N/A
Lighting — Bedroom	Nightlight present but burned out	Replace nightlight	Agreed
Flooring	2 loose area rugs in living room; 1 bath mat (non-slip backing worn)	Remove area rugs; replace bath mat	Patient reluctant about rugs ("they make it feel like home") — educated on fall risk, will discuss with son
Bathroom	Shower chair present (stable); no grab bars near toilet or shower; bath mat worn	Install grab bars at toilet and inside shower; replace bath mat	Son will arrange installation
Bedroom	Hospital bed present (functional); bedside commode in place; path to bathroom clear; 3 pillows for orthopnea positioning	None — adequate	N/A
Kitchen	Moderate clutter on counters; no fire extinguisher; smoke detector present (functional)	Reduce clutter; obtain fire extinguisher; educate on stove safety	Agreed to clutter reduction; son will bring extinguisher
Smoke Detectors	1 in hallway (functional; battery tested); none in bedroom or kitchen	Install smoke detectors in bedroom and kitchen	Son will install
Carbon Monoxide Detector	None	Install CO detector near bedroom	Son will purchase and install
Medications	Stored in kitchen cabinet; not organized; multiple expired medications from prior regimens mixed in; Coumadin not separated from other meds	Organize medications; discard expired meds; separate high-alert medications; place medication list on refrigerator; consider locking med box for Coumadin	RN will organize at next visit with patient/son
Oxygen	Portable concentrator present; tubing adequate length (25 ft); no open flames observed; patient aware of O2 safety (verbal confirmation); no "No Smoking" sign posted	Post "Oxygen in Use — No Smoking" signs at front door and in bedroom	RN will provide signs at next visit

Sharps	None in home at this time	N/A	N/A
Emergency Numbers	Not posted	Post emergency numbers (911, CI after-hours, son Robert, Dr. Chowdhury) on refrigerator	RN will provide card at next visit
Phone Access	Cordless landline in living room; cell phone usually on nightstand	Ensure phone within reach at all times, including bathroom trips	Agreed — will keep cell phone in robe pocket
Weapons	Patient denies firearms or weapons in home	None	N/A
Pets	1 indoor cat — elderly, calm, not a tripping hazard; litter box in laundry room (son cleans weekly)	Monitor cat litter area for fall risk	N/A
General Cleanliness	Adequate; mild clutter in living room (magazines, mail)	Reduce clutter in walkways	Agreed
Temperature	Central heating/cooling present; thermostat set at 72°F; adequate	None	N/A
Water Temperature	Tested at bathroom sink: 118°F	Within safe range (<120°F)	N/A

Overall Home Safety Risk Level: Moderate Immediate Hazards Identified: Loose area rugs (fall risk); absent grab bars at toilet/shower; expired medications mixed with current; no CO detector; no bedroom smoke detector.

SERVICES ORDERED

Service	Discipline	Frequency	Duration	Start Date
Skilled Nursing	RN	3x/week x 3 weeks, then 2x/week x 3 weeks, then 1x/week x 2 weeks	8 weeks (anticipated)	06/19/2025
Physical Therapy	PT	3x/week x 4 weeks, then 2x/week x 4 weeks	8 weeks (anticipated)	06/20/2025
Occupational Therapy	OT	2x/week x 3 weeks, then 1x/week x 3 weeks	6 weeks (anticipated)	06/20/2025
Medical Social Work	MSW	1x/week x 4 weeks, then PRN	4 weeks + PRN	06/23/2025
Home Health Aide	HHA	3x/week (bathing assist, ADL support)	Duration of need	06/23/2025

SKILLED NURSING CARE PLAN FOCUS AREAS

CHF Management & Monitoring

Daily weight monitoring — patient/caregiver education on technique, timing (AM, after void, before breakfast, same clothing), and documentation

Teach CHF "zone" system: Green zone (stable), Yellow zone (call MD — weight gain >2 lbs/day or >5 lbs/week, increased swelling, increased dyspnea), Red zone (call 911 — chest pain, severe SOB, fainting)

Assess bilateral lower extremity edema each visit — measure circumference at ankle and mid-calf

Monitor JVD, lung sounds (crackles), and SpO2 each visit

Reinforce fluid restriction (1.5L/day) — teach fluid measurement and tracking

Reinforce low-sodium diet (2g/day) — provide written materials, review food labels, evaluate frozen meal sodium content

Educate on sodium hidden sources (canned soups, frozen meals, condiments, bread)

Medication Management

Complete medication reconciliation — compare discharge summary, pharmacy bottles, and pill organizer

Dispose of expired/discontinued medications safely

Organize current medications with son/patient

Teach purpose, dose, frequency, timing, side effects, and interactions for all medications, with emphasis on:

Warfarin: INR monitoring schedule, signs of bleeding (bruising, gum bleeding, blood in stool/urine, prolonged wound bleeding, headache), dietary consistency (Vitamin K foods), drug interactions (OTC meds, herbals), lab schedule

Digoxin: teach to check pulse before taking (hold and call MD if <60 or >110 or irregular change), signs of toxicity (nausea, vomiting, visual disturbances, confusion, bradycardia)

Furosemide: timing (not after 2 PM to reduce nocturia), electrolyte monitoring, signs of dehydration vs. fluid overload, importance of potassium supplementation

Carvedilol: orthostatic hypotension precautions, do not abruptly discontinue

Lisinopril: cough as common side effect, report angioedema immediately

Metformin: take with meals, hold if NPO or having contrast procedures, signs of lactic acidosis (rare — muscle pain, weakness, difficulty breathing)

Coordinate with son Robert re: weekly pill organizer filling

Assess need for pharmacy medication synchronization (med sync) program

INR monitoring: draw or coordinate INR draw per physician-specified schedule (goal INR 2.0-3.0 for A-fib)

Oxygen Management

Assess O2 compliance — patient reports removing O2 at night ("it dries out my nose")

Educate on importance of continuous O2 use

Recommend humidifier bottle for concentrator

Verify O2 flow rate matches orders (2 LPM rest, 3 LPM activity)

Assess equipment function at each visit

Monitor SpO2 at each visit (on and off O2 if tolerated)

Reinforce O2 safety (no open flame, no smoking, post signs)

Diabetes Monitoring

FSBS monitoring: patient currently not testing at home (glucometer present but "I forget")

Reinforce daily AM fasting FSBS and PRN if symptomatic

Review FSBS targets per physician (goal 100-180 for this patient given age and comorbidities)

Teach hypo/hyperglycemia signs and management

A1c monitoring: last A1c 7.8% (05/2025) — coordinate with MD for follow-up

Renal Function Monitoring

Monitor for signs of CKD progression: decreased urine output, increased edema, new-onset confusion, nausea

Coordinate lab draws per MD order (BMP/CMP frequency)

Educate patient on CKD and relationship to CHF and DM

Support nephrology referral follow-up

Fall Prevention

Assess fall risk at each visit (current risk: HIGH — history of syncope 05/2025, LE edema, orthostatic BP changes on carvedilol, visual impairment, nocturia, deconditioning)

Reinforce safe transfer techniques

Reinforce call light/phone within reach at all times

Coordinate with PT/OT on fall prevention program

Educate on orthostatic hypotension precautions (rise slowly, sit on edge of bed before standing)

Wound / Skin Surveillance

No wounds currently — monitor for breakdown given edema, CKD, DM, and reduced mobility

Braden Scale reassessment each visit

Educate on skin inspection, moisturizing, and pressure relief

Educate on diabetic foot care (daily inspection, proper footwear, no walking barefoot, no heating pads on feet)

Depression Monitoring

PHQ-9 reassessment every 2 weeks

Assess response to sertraline (started 2023)

Monitor for suicidal ideation

Coordinate with MSW for psychosocial support

Encourage social engagement, activity tolerance progression

Patient/Caregiver Education

All of the above

When to call the MD vs. when to call 911 — provide written wallet card

Teach-back for all education points (CHF zones, weight monitoring, fluid restriction, medications, O2, diet)

Assess and address health literacy limitations (visual impairment, hearing impairment)

PHYSICAL THERAPY EVALUATION FOCUS AREAS

Functional Mobility Assessment

Timed Up and Go (TUG): ____ seconds (anticipated >20 seconds = high fall risk)

30-Second Chair Stand Test: ____ (anticipated <8 = below norm for age)

6-Minute Walk Test: defer until functional status allows; use timed distance test (e.g., 2-minute walk)

Berg Balance Scale: anticipated score <40 (fall risk)

Strengthening Program

Progressive lower extremity strengthening (seated → standing → ambulatory exercises)

Core stabilization

Emphasis on functional task training (sit-to-stand, bed mobility, toilet transfers)

Endurance / Activity Tolerance

Graded activity progression with SpO2 and HR monitoring

Teach Borg Dyspnea Scale for self-monitoring

Collaborate with RN on O2 titration during activity

Gait Training

Rolling walker assessment and training (proper height, technique, turning)

Stair training (2 steps with rail) when safe and appropriate

Obstacle navigation in home

Fall Prevention Program

Balance exercises (static → dynamic)

Environmental modifications (coordinate with OT)

Education on fall recovery techniques

OCCUPATIONAL THERAPY EVALUATION FOCUS AREAS

ADL Training

Energy conservation techniques for bathing, dressing, meal preparation

Adaptive equipment assessment (long-handled shoehorn, sock aid, reacher, tub bench)

Seated task training for kitchen activities

Home Modification Recommendations

Grab bar placement (toilet, shower)

Raised toilet seat assessment

Kitchen organization for accessibility

Medication Management Support

Large-print medication list

Medication organizer setup with visual aids

Adaptive strategies for visual impairment (magnifying glass, contrasting labels)

Energy Conservation / Work Simplification

Activity pacing for CHF management

Teach diaphragmatic breathing during ADLs

Reorganize home to minimize unnecessary ambulation

MEDICAL SOCIAL WORK FOCUS AREAS

Psychosocial Assessment & Support

Depression management — coping strategies, grief counseling referral

Social isolation intervention — community resources, senior center (when mobility improves), phone-based support groups, parish visitor program

Caregiver burden assessment (son Robert)

Community Resources

Meals on Wheels referral (address nutritional needs + sodium-controlled options)

Paratransit enrollment for medical appointments

PG&E Medical Baseline Program enrollment (O2 concentrator)

County emergency services registration

Catholic Charities or parish-based support services

Financial / Insurance Counseling

Verify Medicare/Medi-Cal dual-eligible benefits

Assist with prescription assistance programs if needed

Health Insurance Counseling & Advocacy Program (HICAP) referral for benefits review

Advance Care Planning Support

Review existing AHCD and POLST — confirm patient understands and wishes are current

Facilitate family discussion about goals of care if patient desires

HOME HEALTH AIDE CARE PLAN

Task	Frequency	Specific Instructions
Bathing Assist	3x/week	Seated shower using shower chair; patient washes upper body, aide assists with back, lower body, feet, hair. Water temperature check before bathing. Observe skin for breakdown, redness, new bruising.
Dressing Assist	3x/week (post-bath)	Assist with lower body dressing (socks, shoes — use adaptive equipment as trained by OT). Patient independent with upper body dressing (minimal assist as needed).
Personal Care	3x/week	Nail care (file only — no cutting due to DM and Coumadin); skin moisturizing (lower legs, feet — avoid between toes); oral care assist if needed.
Vital Signs	Each visit	Weight, BP, HR, SpO2. Report to RN: weight gain >2 lbs since last visit, SBP >160 or <90, HR <60 or >110, SpO2 <90%.
Ambulation Assist	Each visit	Assist with ambulation using rolling walker as tolerated. Monitor for dyspnea, fatigue, dizziness.
Light Meal Prep	Each visit (as time allows)	Prepare simple meals per dietary restrictions (low sodium, diabetic). Assist with fluid measurement/tracking.
Environmental Observations	Each visit	Report any safety hazards, changes in home environment, or changes in patient behavior/cognition to RN supervisor.

HHA Supervision: RN supervisory visit every 14 days per 42 CFR §484.80(h). RN must make an on-site visit to patient's home during an HHA visit at least once every 14 days.

PLAN OF CARE GOALS (60-DAY CERTIFICATION PERIOD)

#	Goal Area	Measurable Goal	Target Date
1	CHF Management	Patient/caregiver will verbalize understanding of CHF zones and demonstrate daily weight monitoring with 90% accuracy over 2 consecutive weeks	08/01/2025
2	CHF — Fluid Status	Patient will achieve dry weight of ≤168 lbs (return toward baseline 165 lbs) with no rehospitalization	08/17/2025
3	Medication Management	Patient/caregiver will correctly identify all medications by name, purpose, dose, and key side effects using teach-back with ≥80% accuracy	07/31/2025
4	INR Stability	INR will remain within therapeutic range (2.0-3.0) for ≥80% of draws during episode	Ongoing
5	Oxygen Compliance	Patient will demonstrate continuous O2 use as ordered, including during sleep, with ≥90% compliance per self-report and equipment use data	07/31/2025
6	SpO2 Improvement	SpO2 will be ≥92% on prescribed O2 at rest consistently	08/17/2025
7	Functional Mobility	Patient will ambulate 100 feet with rolling walker and standby assist with SpO2 ≥90%	08/17/2025
8	Transfer Independence	Patient will transfer bed-to-chair with supervision only (no physical assist)	08/01/2025
9	Fall Prevention	Zero falls during episode of care	08/17/2025
10	ADLs	Patient will perform upper body dressing independently and lower body dressing with minimal assist	08/17/2025
11	Nutrition	Patient will verbalize understanding of low-sodium, diabetic-consistent diet and demonstrate ability to identify high-sodium foods with ≥80% accuracy	07/31/2025
12	Depression	PHQ-9 score will decrease to ≤10 (from 14)	08/17/2025
13	Diabetes	Fasting blood glucose will be within target range (100-180) for ≥70% of readings	08/17/2025
14	Skin Integrity	Skin will remain intact with no new pressure injuries, skin tears, or breakdown	08/17/2025
15	Home Safety	All identified safety hazards will be remediated (grab bars, rugs, smoke/CO detectors, medication organization)	07/15/2025

DISCHARGE CRITERIA (ANTICIPATED)

Patient will be considered for discharge when:

CHF is compensated and stable (at or near dry weight, no signs/symptoms of decompensation x 2 consecutive weeks).

Medications are reconciled, organized, and patient/caregiver demonstrates competence in medication management.

INR is stable within therapeutic range.

O2 requirements are stable and patient is compliant.

Functional goals are met or plateau is reached.

Patient/caregiver verbalizes understanding of disease management, dietary restrictions, activity guidelines, and when to seek medical attention.
Home environment is safe.
Community resources are in place (Meals on Wheels, paratransit, etc.).
Follow-up appointments are scheduled with cardiologist, nephrologist, and PCP.
Emotional status is improved and support systems are engaged.

Anticipated Discharge Date: 08/17/2025 (end of first 60-day certification period) — subject to reassessment.

PATIENT / REPRESENTATIVE SIGNATURES — EVALUATION 1

I confirm that the information provided above is accurate and complete to the best of my knowledge. I have participated in the development of this plan of care and agree to the services described.

Patient Signature: _____ Date: _____ Time: _____

Print Name: Margaret Louise Finnegan

Authorized Representative Signature (if applicable): _____

Print Name: Robert Finnegan

Relationship: Son / Health Care Agent (POA)

Witness / Admitting Clinician Signature: _____ Date: _____

Print Name / Title / Credentials: _____

Patient / Representative / Conservator

Mock signature area — training only.

Evaluating Clinician

Mock signature area — training only.

Caregiver / Interpreter if applicable

Mock signature area — training only.

Date / Attestation

Mock signature area — training only.